

7.35.3 NMAC: proposed amendments to the practicum and training provisions

Working draft v6, July 25, 2026. Rule hearing August 28, 2026.

- This document analyzes the recommendation of Dr. Anne Metz to the Training and Education Committee dated July 17, 2026, and states it as amendment language against the proposed rule published July 23, 2026, for the committee's consideration.
- It covers 7.35.3.19, practicum requirements, and the three provisions on which the practicum depends: 7.35.3.18, educational requirements; 7.35.3.14, authorized possession; and Paragraph (5) of Subsection H of 7.35.3.20, staffing ratios. One new section is proposed. No other provision of Part 3 is addressed.
- The Metz recommendation states several requirements as ranges. This draft adopts the low end of each range and marks it, so that the committee may raise it.
- Each proposed change carries a citation to its source. Where the rule as published is unclear, the published text is left as it stands and the question is stated at that provision.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. Metz: 6 to 8 marks a figure taken from the low end of a recommended range.

Hours Published against proposed

COMPONENT	PUBLISHED	PROPOSED	CHANGE
New Mexico module	not stated	6	+6
Psilocybin therapy module, didactic	30	68	+38
Simulated patient experience	5	5	0
Role-specific module	5	5	0
Module total, either permit	40	84	+44
Practicum, facilitator	100	62	-38
Practicum, practitioner	120	72	-48
Supervision or consultation	10	20	+10
Program total, facilitator	150	166	+16
Program total, practitioner	170	176	+6

The practitioner rows take the 20 supervision hours in 7.35.3.19 C to be within the published 120. See 7.35.3.19 C.

7.35.3.14 Authorized possession, purchase, or sale of medical psilocybin

Authorization to possess and administer medical psilocybin. One subsection is added, for training permittees.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in

7.34.3.14 AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY PRACTITIONERS, FACILITATORS, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a practitioner, facilitator, or healing center shall enable practitioners, facilitators, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

Section heading and lead-in

~~7.34.3.14~~ **7.35.3.14** AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY PRACTITIONERS, FACILITATORS, **TRAINING PERMITTEES**, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a practitioner, facilitator, or healing center, **and issuance of a training permit under Subsection G of 7.35.3.19 NMAC**, shall enable practitioners, facilitators, **training permittees**, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

SOURCE Metz recommendation, page 4, training permit. Heading as published reads 7.34.3.14; the history note at the end of the same section reads 7.35.3.14.

Subsection A, Practitioners

(A) Practitioners: Practitioners may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients during administration sessions conducted at a healing center location or other approved location. A practitioner may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

Subsection A, Practitioners

(A) Practitioners: Practitioners may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, during administration sessions conducted at a healing center location or other approved location. A practitioner may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

SOURCE Consequential on proposed 7.35.3.29.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients in administration sessions conducted at healing centers and other approved locations.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, in administration sessions conducted at healing centers and other approved locations.

SOURCE Consequential on proposed 7.35.3.29.

Subsection C, Healing centers (C) Healing centers: Owners and employees of healing centers who are registered with the department may purchase medical psilocybin products from permitted producers, may possess medical psilocybin products, and may sell or otherwise administer those products to qualified patients in administration sessions conducted at the healing center or other approved locations, provided that such individuals are also designated to engage in each activity by the healing center. A healing center may only sell or otherwise provide psilocybin products that are obtained from permitted producers.	Subsection C, Healing centers <i>Not amended.</i> ----- SOURCE Not amended. Please review. This subsection conditions healing center staff authority on being registered with the department. No such registration appears in 7.35.3 NMAC or in 7.35.2 NMAC.
Subsection D, Training permittees <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection D, Training permittees <u>D. Training permittees: A student who holds a current training permit issued under Subsection G of 7.35.3.19 NMAC may possess medical psilocybin products, and may administer or otherwise provide those products to qualified patients, or to practicum participants in accordance with 7.35.3.29 NMAC, in administration sessions conducted at a healing center or other approved location, in each case only while practising under supervision as required by 7.35.3.19 NMAC.</u> ----- SOURCE Metz recommendation, page 4: a training permit "authorizing them to practice under supervision."

7.35.3.18 Educational requirements for certifying clinicians, practitioners, and facilitators

Educational requirements. The 84-hour total is stated, the New Mexico module is given an hour count, and content areas are added to the required topic list.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, practitioner, and facilitator certification: All certifying clinicians, practitioners, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department prior to applying for certification, which shall include at a minimum:

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, practitioner, and facilitator certification: All certifying clinicians, practitioners, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department, consisting of a minimum of six didactic hours, Metz: 6 to 8 prior to applying for certification, which shall include at a minimum: *Paragraphs (1) through (5) not amended.*

SOURCE Metz recommendation, page 2, curriculum table: New Mexico module, 6 to 8 hours.

Please review. The rule sets no date by which the department must create or publish this module. Every certification pathway is conditioned on it.

Subsection B, certifying clinician module

Not amended.

Subsection B, certifying clinician module

Not amended.

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial practitioner and facilitator certification: All practitioners and facilitators shall complete a psilocybin therapy module consisting of a minimum of 30 didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial practitioner and facilitator certification: All practitioners and facilitators shall complete a psilocybin therapy module consisting of a minimum of ~~30~~ **68** didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

SOURCE Arithmetic. Metz recommendation, page 1, states a total of 84 hours. 84 less Subsection A (6), less Subsection D or E (5, unchanged), less the simulated patient experience (5, unchanged) leaves 68.

Please review. 68 follows from the 84-hour total after Subsection A, Subsection D or E, and the simulated patient experience. If any of those three changes, 68 changes.

Subsection C, required topics

- (1) Overview of practitioner/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) A simulated patient experience of no less than 5 hours; and
- (17) Evaluation to demonstrate competency in the above areas.

Subsection C, required topics

- (1) Overview of practitioner/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care;
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) End-of-life and palliative care;
- (17) Screening, suicidality, and crisis response;
- (18) Substance use disorder;
- (19) Post-traumatic stress disorder;
- (20) Somatic awareness and therapeutic touch;
- (21) History and traditional use of psilocybin and other psychedelics;
- (22) Individual and group treatment models;
- (23) Challenging experiences, including hallucinogen persisting perception disorder;
- ~~(16)~~ (24) A simulated patient experience of no less than 5 hours; and
- ~~(17)~~ (25) Evaluation to demonstrate competency in the above areas.

SOURCE Metz recommendation, pages 2 and 3, curriculum table, for paragraphs (16), (17), (20), (21), (22) and (23). July 17, 2026 Training and Education Committee transcript for paragraphs (18) and (19); both are qualifying conditions under Section 3(l) of the Medical Psilocybin Act.

Please review. The Metz recommendation gives an illustrative range for each content area and states that the binding minimum is the total, so no per-topic minimum is drafted. Ranges: core psychotherapy skills and ethics 25 to 30; end-of-life and palliative care 10 to 15; trauma 8; medicine, dosing and clinical research literacy 8 to 10; New Mexico module 6 to 8; treatment models 6 to 8; screening, suicidality and crisis response 5 to 8; somatic awareness and therapeutic touch 4 to 6; history and traditional use 3 to 4; challenging experiences 2 to 4. Should any be drafted as a minimum? No hour figure is given for post-traumatic stress disorder or substance use disorder.

Subsection D, additional facilitator module <i>Not amended.</i>	Subsection D, additional facilitator module <i>Not amended.</i>
Subsection E, additional Practitioner module <i>Not amended.</i>	Subsection E, additional Practitioner module <i>Not amended.</i>
Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner and facilitator trainings: All practitioners and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in:	Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner and facilitator trainings: All practitioners and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in: <u>complete and maintain the following prior to applying for medical psilocybin certification:</u> Paragraphs (1) and (2) not amended. Corrects a sentence that does not complete as published. No obligation changes. ----- SOURCE Correction. The sentence as published does not complete.
Subsection G, continuing education <i>Not amended.</i>	Subsection G, continuing education <i>Not amended.</i>
Subsection H, total module hours <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection H, total module hours <u>H. Total module hours: The requirements of Subsections A, C and D of this section shall together total a minimum of 84 hours for a facilitator applicant. The requirements of Subsections A, C and E of this section shall together total a minimum of 84 hours for a practitioner applicant. The hours stated for each module in this section are minimums within that total.</u> ----- SOURCE Metz recommendation, page 3: “Component ranges are illustrative; the binding minimum is the total.”

7.35.3.19 Practicum requirements for practitioners and facilitators

Practicum requirements. Practicum totals, the supervision hours, and the practicum sequence are amended; the training permit, the consultation requirement and the end-of-life checkpoint are added.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a practitioner or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of 100 hours of supervised practice training for facilitators and 120 hours for practitioners and shall be completed prior to applying for certification. Students shall participate in a minimum of 80 hours of administration day sessions, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria:

Subsection B, minimum practicum hours; preparation and integration sessions

B. Minimum practicum hours; in-person and integration sessions: Students shall participate in a minimum of 20 hours of in-person preparation and integration sessions, where students are provided the opportunity to experience, observe, or conduct (when licensure allows) preparation or integration sessions. This shall include a minimum of six different patients during individual sessions. This shall include at least one group preparatory and one group integration session with a minimum of four or more patients.

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a practitioner or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of ~~100~~ 62 Metz: approx. 62 hours of supervised practice training for facilitators and ~~120~~ 72 Metz: approx. 72 hours for practitioners and shall be completed prior to applying for certification. Students shall participate in ~~a minimum of 80 hours of administration day sessions, where students are~~ administration day sessions as required by Subsection D of this section, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria: *Paragraphs (1) and (2), the patient and session minimums, not amended.*

SOURCE Metz recommendation, page 4: “approximately 62 hours for Facilitators (steps 1-3); approximately 72 hours for Licensed Providers (steps 1-4).”

Please review. Two points. The published phrase “half of the didactic requirements” does not identify half of what. The published figure of 80 hours of administration day sessions cannot sit within a 62-hour practicum, so it is struck and the step hours in Subsection D govern; the minimums of 14 different patients over eight different sessions are carried forward and should be confirmed as achievable in 62 hours.

Subsection B, minimum practicum hours; preparation and integration sessions

Not amended.

SOURCE Not amended.

Subsection C, Practitioner supervision hours

C. Practitioner supervision hours: Practitioners shall complete an additional minimum of 20 hours as a practitioner supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group.

Subsection C, Practitioner supervision hours

C. Practitioner supervision hours: Practitioners shall complete an additional minimum of ~~20~~ **10** **Metz: 10** hours as a practitioner supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group.

SOURCE Metz recommendation, page 4, step 4: "Supervisory hours, Licensed Providers only (10 hours)."

Please review. Subsection A sets 120 hours for a practitioner. This subsection adds "an additional minimum of 20 hours" without identifying what it is additional to. Within the 120, the published practitioner total is 170. On top of the 120, it is 190. The Metz totals of approximately 62 and 72 differ by exactly her 10 supervisory hours, so the recommendation treats them as within. This draft follows the recommendation, giving a proposed total of 176. Which reading governs?

Subsection D, practicum sequence

There is no current language. This provision does not exist in the proposed rule as published.

Subsection D, practicum sequence

D. Practicum sequence: The practicum required by Subsection A of this section consists of the following, in sequence:

(1) approximately 30 hours of initial facilitation experience. This stage shall be completed with practicum participants who are not qualified patients, in accordance with 7.35.3.29 NMAC. Preparation and integration sessions are required for each participant;

Metz: approx. 30

(2) approximately 20 hours of co-facilitated cases, being a minimum of two cases co-facilitated with a certified practitioner or facilitator, including administration sessions and the associated preparation and integration sessions. The student holds a training permit issued under Subsection G of this section at this stage and serves as the second person in the co-facilitation pair; **Metz: approx. 20**

(3) approximately 12 hours of group practicum, being direct participation in group preparation, administration, and integration sessions; and **Metz: approx. 12**

(4) for practitioner applicants only, the supervision hours required by Subsection C of this section.

SOURCE Metz recommendation, pages 3 and 4, steps 1 through 4.

Subsection E, practicum location (published Subsection D) D. Practicum location: All practicum hours shall take place in an approved healing center or other approved location.	Subsection E, practicum location (published Subsection D) D. E. Practicum location: All practicum hours shall take place in an approved healing center or other approved location. <u>An approved supervisor at an approved location may host practicum students who are not enrolled at a co-located training program.</u> SOURCE Metz recommendation, page 4: “an approved supervisor at an approved location can host practicum students who are not enrolled at a co-located training program.”
Subsection F, practicum standards (published Subsection E) E. Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice.	Subsection F, practicum standards (published Subsection E) E. E. Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice. <i>Re-lettered only. Text not amended.</i> SOURCE Not amended. Re-lettered.
Subsection G, training permit <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection G, training permit <u>G. Training permit: On completion of the stage described in Paragraph (1) of Subsection D of this section, the department shall issue to the student a time-limited training permit authorizing the student to practise under supervision, first as the second member of a co-facilitation pair during the stages described in Paragraphs (2) and (3) of Subsection D of this section, and then while completing the requirements of Subsection H of this section. The full facilitator or practitioner certification issues only when all requirements are met.</u> SOURCE Metz recommendation, page 4, training permit. Please review. The Metz recommendation states when the permit issues and what it authorizes. It states no term, renewal period, or application process, and none is drafted.

Subsection H, supervision and consultation; case presentation sign-off <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection H, supervision and consultation; case presentation sign-off <u>H. Supervision and consultation; case presentation sign-off:</u> <u>(1) A training permittee shall complete a minimum of 20 hours of supervision or consultation during the training permit period, in addition to the practicum hours required by this section;</u> Metz: 20 to 30 <u>(2) sign-off is tied to case presentation. The permittee shall present a minimum of two cases of clients the permittee has personally worked with using regulated medicine. Each case presentation shall take the form of a biopsychosocial case conceptualization, including a discussion of presenting concerns, risk factors, supportive factors, treatment considerations, and recommendations for aftercare; and</u> <u>(3) the consultant or supervisor shall complete a standardized evaluation form on each case presented.</u> ----- SOURCE Metz recommendation, pages 4 and 5: 20 to 30 hours of supervision or consultation, sign-off tied to two presented cases with a standardized evaluation form.
Subsection I, end-of-life practice <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection I, end-of-life practice <u>I. End-of-life practice: Before serving end-of-life participants independently, a practitioner or facilitator shall complete at least one co-facilitated end-of-life case and present at least one end-of-life case in supervision or consultation.</u> ----- SOURCE Metz recommendation, page 5, end-of-life checkpoint.
Subsection J, waiver of practicum requirements (published Subsection F) F. Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program.	Subsection J, waiver of practicum requirements (published Subsection F) F. <u>J</u>. Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program. <i>Re-lettered only. Text not amended.</i> ----- SOURCE Not amended. Re-lettered.

Subsection K, waiver for applications received by December 31, 2027 (published Subsection G)

G. Waiver of practicum hours requirement for applications received by December 31, 2027: An applicant for certification shall not be required to satisfy the full New Mexico practicum hours requirement if the applicant:

- (1) Applies for certification by December 31, 2027;
- (2) Completes the didactic requirements by December 31, 2027;
- (3) Graduates from an educational program that the department certifies by December 31, 2027 or that the department has included on the department-approved list of educational programs by December 31, 2027; and
- (4) Demonstrates completion of at least 40 hours of contact time through logs or other records of the sessions, including:
 - (a) A minimum of two separate individual sessions including the appointments for preparation, administration and integration; and
 - (b) A minimum of one group session including the appointments for preparation, administration, and integration.

Subsection K, waiver for applications received by December 31, 2027 (published Subsection G)

~~G.~~ **K.** Waiver of practicum hours requirement for applications received by December 31, 2027: *Re-lettered only. Paragraphs (1) through (4) not amended.*

SOURCE Not amended. Re-lettered.

Please review. Paragraph (4)(b) requires a minimum of one group session. Subsection D(1) of 7.35.3.10 requires two separate group sessions for the same 40 hours of contact time.

7.35.3.20 Requirements for healing centers and other approved locations

Requirements for healing centers and other approved locations. Paragraph (5) of Subsection H, the staffing ratio, is the only paragraph amended.

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Subsection H(5), staffing ratios (5) Includes procedures to ensure that there are practitioners and facilitators present at all times during an administration session with a minimum of one practitioner and one facilitator for individual patient sessions; and in group administration sessions a minimum of one practitioner for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated.	Subsection H(5), staffing ratios (5) Includes procedures to ensure that there are practitioners and facilitators present at all times during an administration session with a minimum of one practitioner and one facilitator for individual patient sessions; and in group administration sessions a minimum of one practitioner for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program<u>hold a current training permit issued under Subsection G of 7.35.3.19 NMAC</u> and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated. ----- SOURCE Consequential on the training permit at proposed 7.35.3.19 G. The 50-hour threshold is as published.

7.35.3.29 Proposed new section. Practicum participants who are not qualified patients

Proposed new section. The first stage of the practicum, conducted with participants who are not qualified patients.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, practicum participants who are not qualified patients

There is no current language. This provision does not exist in the proposed rule as published.

Entire section, practicum participants who are not qualified patients

7.35.3.29 PRACTICUM PARTICIPANTS WHO ARE NOT QUALIFIED PATIENTS:
A. Purpose: This section provides for the stage of the practicum described in Paragraph (1) of Subsection D of 7.35.3.19 NMAC to be conducted with practicum participants who are not qualified patients, so that a student gains supervised exposure to non-ordinary states before working with clinical populations.
B. Practicum participant: A practicum participant is an individual who is not a qualified patient and who takes part in a practicum session in a retreat or peer-support model. Preparation and integration sessions are required for each participant.
C. Effect: This section applies only to the extent that the Medical Psilocybin Act, Sections 26-2D-1 through -11 NMSA 1978, authorizes the administration of medical psilocybin to a person who is not a qualified patient.

SOURCE Metz recommendation, page 3, step 1: "Initial facilitation experience with well participants (approximately 30 hours). Two to three sessions as a facilitator in a retreat or peer-support model."

Please review. Section 5(B)(2) of the Medical Psilocybin Act reaches "a clinician administering or a qualified patient taking psilocybin in an approved setting." Section 3(H) defines a qualified patient by diagnosis with a qualifying condition. A participant who is not a qualified patient falls outside both. Subsection C conditions this section accordingly.

Addendum A Provisions on which the practicum operates

Provisions on which 7.35.3.19 operates, and whether this draft amends each.

Provision	Page	What the practicum needs from it	In this draft
7.35.2.7 NMAC	n/a	Supplies every defined term used in Part 3, by force of 7.35.3.7	Not amended. Facilitator, healing center, certifying clinician and student are used in Part 3 and defined in neither part. Addendum B
Medical Psilocybin Act, Section 5	n/a	The criminal and civil exemption. Names producers, clinicians and qualified patients, and protects presence only for anyone else	Statute. Not reachable by rule
7.35.3.9 C(3), E, F	3	Certification applications require documentation of the completed practicum, and E(1) sets the practitioner licensure predicate	Not amended
7.35.3.10 D	5	Out-of-jurisdiction 40-hour practicum waiver	Not amended. Disagrees with 7.35.3.19 K
7.35.3.11 A, B	5 to 7	Healing centers and other approved locations. The practicum may only happen in them	Not amended
7.35.3.12 A	7	Educational program certification	Not amended
7.35.3.13 B	8	Facilitator scope of work, which limits what the practicum can usefully train	Not amended
7.35.3.14	9	Authority to possess and administer	AMENDED. New Subsection D
7.35.3.15 A(4)	9	Practicum site agreements need prior written approval, with no deadline on the department	Not amended
7.35.3.17 A	10 to 11	The 10 mentoring hours the consultation requirement would replace	Not amended
7.35.3.17 B, C	11	Test-out, graduation conditions, and the practicum records a program must keep	Not amended
7.35.3.18	11 to 12	The didactic requirement the practicum entry gate is measured against	AMENDED
7.35.3.19	12 to 13	The practicum	AMENDED. Re-lettered A to K
7.35.3.20 D	14	Students may be present at an administration session	Not amended
7.35.3.20 H(5)	14	Staffing ratio, which counts a qualified student	AMENDED
7.35.3.22	15	Prohibitions, reaching "the qualified patient or certificant"	Not amended
7.35.3.27	16 to 19	Discipline and appeal	Not amended

Proposed new provisions

Provision	What it does	Source	If dropped
7.35.3.14 D	Authorizes a training permittee to possess and administer under supervision	Recommendation 3, page 4	The practicum stays unauthorized. Stands or falls with 7.35.3.19 G
7.35.3.18 H	States the 84-hour total in one place	Recommendation 2, page 3	Module hours survive individually; no total is stated anywhere
7.35.3.19 D	The four-step practicum sequence	Recommendation 3, pages 3 to 4	Practicum totals survive; the sequence is lost
7.35.3.19 G	Creates the training permit	Recommendation 3, page 4	The practicum stays unauthorized
7.35.3.19 H	20 consultation hours, two presented cases, standardized evaluation form	Recommendation 4, pages 4 to 5	7.35.3.17 A survives at 10 hours with no requirement to have seen a client
7.35.3.19 I	End-of-life checkpoint	Recommendation 4, page 5	No end-of-life competency gate remains
7.35.3.29	Practicum participants who are not qualified patients	Recommendation 3, step 1, page 3	The first stage of the practicum has no lawful setting

Addendum B Defined terms and the permit title

7.35.3.7 provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." 7.35.2 NMAC was adopted effective June 23, 2026. The terms below are used in Part 3. No amendment to either part is proposed in this document.

Term used in Part 3	In 7.35.2.7?	What 7.35.2.7 has	Consequence
Facilitator	No. Zero occurrences in 7.35.2 NMAC	"Guide" an individual who has completed training and education approved by the department to be able to assist practitioners during the administration sessions and who has been registered with the department	Every facilitator provision in Part 3 rests on an undefined term. A guide holds no professional license, so a facilitator is not a "clinician" under Section 3(B) of the Act, while 7.35.3.14 B authorizes facilitators to possess and provide
Healing center	No	"Approved location" means a location approved by the department for psilocybin administration sessions	Healing centers are certified and regulated throughout Part 3 with no definition
Certifying clinician	No	"Clinician" means an approved health care provider licensed in New Mexico who holds a certification from the department to provide medical services to qualified patients	Probably the same role renamed
Student	No	Nothing	Carries consequence at 7.35.3.19, 7.35.3.20 D and 7.35.3.20 H(5)
Practitioner	Yes	"Practitioner" means an individual who is a licensed healthcare professional who is certified by the department to provide medical psilocybin integrative therapy, supervise guides, and who has completed department required trainings	Carries the same licensure predicate as the Act's "clinician"

The permit title. Recommendation 1 proposes retitling "Practitioner" as "Licensed Provider". This draft keeps "practitioner" because the decision has not been made. The title is held as a variable in the source, so changing it is one command. It reaches no hour count and no practicum requirement: those attach to the permit type, and what the practicum is turns on the licensure predicate at 7.35.3.9 E(1), which already requires a New Mexico professional license. A retitle is an amendment to 7.35.2.7 NMAC plus a conforming pass over Part 3.

Addendum C Sections not addressed

Part 3 has twenty-eight sections. This draft addresses the practicum and the provisions on which it operates. The sections below are not addressed. Findings are recorded in the concerns inventory of July 23, 2026, which lists five blocking and twenty-three material findings across the part.

Group	Sections	Recorded findings
Terminology	7.35.3.7, and 7.35.2.7 NMAC	Facilitator, healing center, certifying clinician and student undefined. Addendum B
Educational programs	7.35.3.12, .15, .16, .17	The evaluator conflict rule disqualifies the evaluators a program must hire; the third-party evaluation deferral expires the day it becomes available; no deadline binds the department on instructor changes; the denial grounds were deleted; the evaluator qualification standard may be unmeetable; mentoring is unfunded and untied to certification; the test-out price cap assumes per-module pricing
Out-of-jurisdiction pathway	7.35.3.10	The two 40-hour waivers disagree; the 2027 waiver may cancel itself; the approved list is empty at launch and populates only retroactively; individuals must produce an evaluation only a program can commission
Locations and oversight	7.35.3.11, .20, .21	Healing center staff authority hangs on a registration that does not exist; outdoor locations need no AED while healing centers do; the patient roster and the patient-interview authority; other approved locations have a 90-day term and no renewal path
Patients, applicants, process	7.35.3.8, .9, .13, .22 to .27	Facilitator scope conflicts with the practicum and the ratios; complainants lose confidentiality; the benefit-risk attestation; renewals can lapse while pending; the re-application clause reads as a deadline; two review tracks with inconsistent scope; a suspended certificant can be out of practice more than 135 days
Mechanical	Throughout	Five section headings read 7.34.3 instead of 7.35.3, two of which are corrected here; two sections carry a real effective date while the rest carry placeholders; a numbering gap at 7.35.3.9 D

Sources. Rule as published: *rules-draft-2026-07-23-published.pdf*, 19 pages. Recommendations: Dr. Anne Metz, *Recommendations on Education and Training Requirements for Facilitators and Licensed Providers*, July 17, 2026, with the one-page summary and the committee slide deck of the same date. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION." Medical Psilocybin Act, Senate Bill 219, 2025, as introduced; the enacted text at Sections 26-2D-1 through -11 NMSA 1978 has not been checked against it. 7.35.2 NMAC as adopted effective June 23, 2026.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and no other alteration. Each block was checked against the text layer of the published rule.

Status. Working draft v6. Not a filing, not submitted, and not adopted rule text.